Nasogastric Intubation

1) Introduce yourself – medical student
2) Introduce procedure - risks & benefits ⟶ brief pt. about complications & indications (purpose)
3) Receive informed consent
4) Ensure all equipments are ready
5) Wash your hands, glove up
6) Measure the length of tube (ear lobe → tip of nose → epigastrium area)
7) Place patient in 30-45° position
8) Lubricate the end of the tube
9) Insert the tube up the pt's nose until you hit the nasopharynx
10) Ask the pts. to start swalling (w/ water) so tube goes to oropharynx
    ↳ can also ask pt. to swallow saliva
11) Check if the tube is in the right position
    ↳ Aspirate stomach content (pull syringe)
    ↳ Push air into epigastrium via syringe into the tube & listen w/ stethoscope
    ↳ can check w/ radiology (uncommon)
12) Stabilize the tube w/ tape at the nose
13) Check if the patient can talk

Indications:
1) Aspirate gastric content for lab
    ↳ Detect GI bleed, gastric lavage content (aspirate stomach content & replace w/ saline), pathogen detection
2) Reduce gastric distension
3) Feeding

Complication:
1) Aspiration (สำลัก)
2) Skin lesion at nose → abrasions
3) Infection (sinus infection, middle ear infection)

Contraindication:
1) Esophagus injury due to ingestion of corrosive agents
2) Oil content / hydrocarbon in stomach ⟶ lipoid pneumonia (lipid content enters cheek)
3) Base of skull fracture

Red = largest diameter (18 Fr) ⟶ Larger individuals
Orange = greater diameter (16 Fr) ⟶ Men
Green = smallest diameter (14 Fr) ⟶ Women
} There are smaller sizes for children